

PATHOLOGY LABORATORY REPORT

St. Bartholomew Medical Centre — Lab Division

PATIENT NAME: John
Patterson
DATE OF BIRTH:
12/04/1979 (Age: 47
years)
REFERRED BY: Dr.
Samantha Reed
REPORT DATE: 01 May
2026

PATIENT ID:
PT-2026-07841
GENDER: Male

COLLECTION DATE: 01
May 2026
SPECIMEN: Venous
Blood (EDTA + SST)

COMPLETE BLOOD COUNT (CBC)

TEST	RESULT	UNIT	REFERENCE RANGE	FLAG
Hemoglobin (Hb)	18.4	g/dL	13.5 – 17.5	HIGH
Red Blood Cells (RBC)	6.1	×10 ⁶ /µL	4.5 – 5.9	HIGH
White Blood Cells (WBC)	11.9	×10 ³ /µL	4.5 – 11.0	HIGH
Neutrophils	78	%	40 – 70	HIGH
Lymphocytes	14	%	20 – 40	LOW
Monocytes	6	%	2 – 10	
Eosinophils	2	%	1 – 6	
Platelets	138	×10 ³ /µL	150 – 400	LOW
Hematocrit (PCV)	54	%	41 – 53	HIGH
MCV	88	fL	80 – 100	
MCH	30	pg	27 – 33	
MCHC	34	g/dL	31.5 – 36	

COMPREHENSIVE METABOLIC PANEL

TEST	RESULT	UNIT	REFERENCE RANGE	FLAG
Glucose (Fasting)	126	mg/dL	70 – 99	HIGH
HbA1c	7.2	%	< 5.7%	HIGH
Creatinine	1.4	mg/dL	0.7 – 1.3	HIGH
Blood Urea Nitrogen (BUN)	22	mg/dL	7 – 20	HIGH
eGFR	58	mL/min/1.73m ²	> 60	LOW
Sodium (Na ⁺)	139	mEq/L	136 – 145	
Potassium (K ⁺)	3.3	mEq/L	3.5 – 5.1	LOW
Chloride (Cl ⁻)	101	mEq/L	98 – 107	

Bicarbonate (HCO_3^-)

mEq/L

			22 – 29	
ALT (SGPT)	52	U/L	7 – 40	HIGH
AST (SGOT)	48	U/L	10 – 40	HIGH
Total Bilirubin	1.1	mg/dL	0.2 – 1.2	
Albumin	4.1	g/dL	3.5 – 5.0	

LIPID PROFILE

TEST	RESULT	UNIT	REFERENCE RANGE	FLAG
Total Cholesterol	242	mg/dL	< 200	HIGH
LDL Cholesterol	162	mg/dL	< 130	HIGH
HDL Cholesterol	38	mg/dL	> 40	LOW
Triglycerides	208	mg/dL	< 150	HIGH
VLDL Cholesterol	42	mg/dL	< 30	HIGH
LDL/HDL Ratio	4.3		< 3.5	HIGH

IRON STUDIES

TEST	RESULT	UNIT	REFERENCE RANGE	FLAG
Serum Iron	180	µg/dL	60 – 170	HIGH
Total Iron Binding Capacity	290	µg/dL	250 – 370	
Transferrin Saturation	62	%	20 – 50	HIGH
Ferritin	680	ng/mL	12 – 300	HIGH

CLINICAL COMMENTS

Significant findings: Polycythemia (elevated Hb, RBC, Hematocrit) with leukocytosis and thrombocytopenia. Iron studies consistent with iron overload / possible Polycythemia Vera. Pre-diabetic state indicated by fasting glucose 126 mg/dL and HbA1c 7.2%. Dyslipidemia with elevated LDL, low HDL, elevated triglycerides — increased cardiovascular risk. Mildly impaired renal function (eGFR 58) and mildly elevated liver enzymes (ALT, AST). Recommend haematology referral, repeat TFTs, JAK2 mutation screen, and renal function follow-up in 4 weeks.

